PATIENT CLINICAL CONSULTATION AGENDA & SYMPTOM INVENTORY

Structured Patient Experience (Plain Language) & Pathophysiology for Bedside Review | Wednesday, 4 November 2026

Patient: James Jones	**DOB:** 20/01/1986 (Age: 40)	**Medicare:** 3634 32502 1 (Ref: 1, Exp: 07/2032)
Centrelink CRN: 303065993H	**Consult Date:** Wednesday, 4 November 2026	**Clinic:** Dr. Nathan Anderson's Clinic (GP Resident)

Key Bedside Priorities: (1) Acute Motor Deficit: Right foot drop with daily tripping / fall risk onto fused cervical spine (urgent AFO prescription). **(2) Neuro-Axis Discordance:** Severe patellar hyperreflexia co-exacerbated during acute sciatica flares (cervical myelomalacia UMN vs. lumbar LMN). **(3) Legal & NDIS Documentation:** Clinic medical certificate for police/bail attendance compliance + NDIS Access Part B sign-off / summary letter.

Item 1: Hyperreflexia & Patellar Kick (Worse When Sciatica Flares)

Patient Experience (Subjective): My knee kicks out violently and uncontrollably when touched or lightly tapped. This exaggerated kick gets noticeably worse whenever my lower back sciatica pain flares up, and when the knee is hyper-reactive my back pain also feels more intense.

Clinical Correlation & Pathophysiology: Bilateral patellar hyperreflexia (Grade 3+ to 4+ deep tendon reflex with unsustained clonus). Upper Motor Neuron (UMN) corticospinal tract disinhibition secondary to pre-existing cervical myelomalacia (C5–C7). Paradoxical co-exacerbation: acute lumbar nociceptive barrage cross-excites spinal interneuron reflex arcs (neuro-axis central sensitization).

Required Consult Action / Objective: Bedside reflex hammer testing of patellar and Achilles reflexes; clinical evaluation of Upper Motor Neuron (cervical cord) vs Lower Motor Neuron (lumbar root) interaction.

Item 2: Provocative Sciatic Nerve Shockwaves on Forward Lean

Patient Experience (Subjective): When I lean forward even slightly with a bent knee, a violent electric shock shoots up the back of my leg directly into the disc bulge in my lower back.

Clinical Correlation & Pathophysiology: Dynamic neurodynamic tension / slump test positive. Provocative dural and sciatic nerve root traction shockwaves confirming acute mechanical space-occupying disc protrusion (L4–L5 / L5–S1) impinging on the traversing/exiting nerve root.

Required Consult Action / Objective: Physical examination of neurodynamic tension; review of lumbar mechanical compression and disc stabilization.

Item 3: Right Foot Drop & Daily Tripping (Loss of Ground Clearance)

Patient Experience (Subjective): I cannot lift my right foot properly when walking. My toes drag and catch on carpets and floors, forcing me to lift my knee high (high-stepping), leading to daily stumbling and near-falls.

Clinical Correlation & Pathophysiology: Acute Lower Motor Neuron (LMN) motor deficit: marked weakness of right tibialis anterior and extensor hallucis longus (Grade 3/5 dorsiflexion). Unilateral right foot drop, loss of ground clearance in swing phase, and antalgic high-steppage gait.

Required Consult Action / Objective: Formal documentation of acute motor deficit in medical record; urgent referral / prescription for Ankle-Foot Orthosis (AFO) splint to prevent catastrophic falls onto fused cervical spine.

Item 4: Contralateral Left C7–T1 Burning Pain on Raising Right Arm

Patient Experience (Subjective): Whenever I lift, reach out, or raise my RIGHT arm overhead, I feel a sharp, hot burning pain on the LEFT side of my neck and upper back near my shoulder blade.

Clinical Correlation & Pathophysiology: Contralateral cervicothoracic junction (C7–T1) neuropathic burning evoked by contralateral arm scaption/abduction. Rotational shear stress and scapular anchoring forces transfer across the thoracic ribcage and dump into the rigid transitional C7–T1 segment immediately adjacent to the C5–C7 fusion construct.

Required Consult Action / Objective: Cervicothoracic and scapulothoracic kinetic chain examination; adjacent segment strain assessment.

Item 5: White, Numb Fingers When Cold (Raynaud's / Acral Vasospasm)

Patient Experience (Subjective): My index and middle fingers suddenly turn chalk-white, freezing cold, and completely numb when exposed to cold air or chilly weather.

Clinical Correlation & Pathophysiology: Cold-induced acral vasospasm / secondary Raynaud's phenomenon. Marked digital ischemic blanching and paresthesia, likely driven by autonomic sympathetic chain irritation at the cervicothoracic junction vs microvascular dysregulation.

Required Consult Action / Objective: Microvascular and peripheral pulse check; evaluation of sympathetic involvement related to cervical spine pathology.

Item 6: Morning Lumbar Stiffness, Evening Muscle Spasms & Sleep Disruption

Patient Experience (Subjective): I wake up with a completely rigid lower back. As the day goes on, standing and sitting exhaust my muscles, turning into painful, exhausting spasms by night that wake me up repeatedly.

Clinical Correlation & Pathophysiology: Severe diurnal paraspinal guarding and muscular fatigue. Early morning mechanical stiffness (VAS 6.8/10) escalating under upright axial gravity loading into severe evening paraspinal spasms (VAS 7.7/10) by 21:00, causing severe sleep fragmentation.

Required Consult Action / Objective: Review of daytime pacing protocols, muscle spasm pharmacotherapy, and sleep stabilization.

Item 7: Right Anterior Shoulder & Tendon Impingement on Elevation

Patient Experience (Subjective): Pain in the front of my right shoulder when lifting my arm out to the side or overhead, feeling like a tendon is catching or pinching.

Clinical Correlation & Pathophysiology: Right anterior shoulder subacromial impingement arc (60°–120° scaption) and bicipital/supraspinatus tendinopathy. Secondary to chronic altered scapulothoracic kinematics and persistent trapezius guarding following cervical spine fusion.

Required Consult Action / Objective: Neer's and Hawkins-Kennedy impingement testing; clinical differentiation from C6 radicular shoulder pain.

Item 8: Physical Inability to Handwrite & Loss of Fine Motor Dexterity

Patient Experience (Subjective): I physically cannot write with a pen for more than a minute. Holding a pen triggers severe neck cramps, numbness shooting down my arm, and my fingers lock up and lose control.

Clinical Correlation & Pathophysiology: Loss of fine motor dexterity and rapid intrinsic hand muscle fatigue secondary to cervical myelomalacia and C7/C8 nerve root tension. Static neck flexion combined with sustained pincer grip induces acute cervicobrachial spasm and functional handwriting incapacity.

Required Consult Action / Objective: Document physical handwriting incapacity in medical notes as clinical justification for digital attachments on legal and NDIS documents.

Item 9: Cervicobrachial Radiculopathy & Arm Numbness / Neck Tightness

Patient Experience (Subjective): Constant tightness and burning in the back of my neck and shoulder blade, with pins-and-needles tingling traveling down the back of my arm into my fingers.

Clinical Correlation & Pathophysiology: Chronic cervicobrachial radiculopathy (predominantly C7 dermatomal distribution). Persistent posterior cervical myofascial guarding, upper trapezius trigger bands, and triceps/dorsal forearm paresthesia.

Required Consult Action / Objective: Spurling's test and upper limb dermatomal/myotomal sensory testing.

Item 10: Intractable 24/7 Deep Right Lower Back & Leg Pain (Sciatica)

Patient Experience (Subjective): A deep, agonizing ache and burning in my right lower back, buttock, and leg that never stops—it is there 24 hours a day, 7 days a week, and is my biggest source of pain.

Clinical Correlation & Pathophysiology: Intractable right L4–L5 / L5–S1 lumbar radiculopathy and discogenic somatic pain. Primary continuous driver of physical disability (62% anatomical burden in longitudinal logs; baseline VAS 7.4/10).

Required Consult Action / Objective: Review of interventional pain management plan; review of candidacy for Spinal Cord Stimulation (SCS) with Dr. Brett Todhunter.

Item 11: Mid-Thoracic & Interscapular Back Pain

Patient Experience (Subjective): A constant dull ache and heavy fatigue in the middle of my back between my shoulder blades that gets worse when sitting upright.

Clinical Correlation & Pathophysiology: Mid-thoracic (T4–T8) axial muscular strain and costovertebral joint dysfunction. Postural compensatory fatigue resulting from altered sagittal balance between the rigid fused cervical spine above and the degenerative lumbar spine below.

Required Consult Action / Objective: Thoracic spinal palpation and rib cage mobility assessment.

Item 12: Doctor's Medical Certificate for Police & Bail Compliance

Patient Experience (Subjective): I need a formal medical certificate for today for police/bail proving that I attended the medical clinic and that my spinal disability physically prevents standing in lines or attending in person.

Clinical Correlation & Pathophysiology: Issuance of formal medical certificate certifying clinical attendance, acute spinal fragility, severe mobility restriction, and medical contraindication to prolonged static standing or physical travel for legal and bail compliance.

Required Consult Action / Objective: Complete and print formal medical certificate for police/court records today.

Item 13: NDIS Access Request Form (Part B Sign-Off or 1-Page Summary Letter)

Patient Experience (Subjective): I need the doctor's section of the NDIS form signed (or a 1-page summary letter on clinic letterhead) confirming my spinal conditions are permanent and substantially reduce my ability to walk, care for myself, and write.

Clinical Correlation & Pathophysiology: Treating Health Professional certification for NDIS Access Request (Section 24 criteria). Documentation confirming permanent diagnoses (C5–C7 ACDF with myelomalacia + L4–S1 disc extrusion with foot drop), permanence of impairment, and substantial reduction in functional capacity in Mobility, Self-Care, Self-Management, and Communication. (Fallback: 1-page clinical summary letter).

Required Consult Action / Objective: Sign Part B of NDIS Access Form OR provide signed 1-page clinical summary letter on clinic letterhead.

Item 14: Prescription / Referral for Ankle-Foot Orthosis (AFO Splint)

Patient Experience (Subjective): I need a prescription or referral for a foot-up splint (AFO brace) to keep my right foot from dragging so I do not trip and fall, which could severely damage my neck fusion.

Clinical Correlation & Pathophysiology: Medical prescription or allied health / orthotist referral for an Ankle-Foot Orthosis (AFO) or dynamic foot-up splint for mechanical gait correction of right tibialis anterior weakness and fall prevention.

Required Consult Action / Objective: Provide prescription or referral letter for orthotist / AFO fitting.

Item 15: Clinical Query: Do I Need Another MRI? Is It Definite Cord Compression?

Patient Experience (Subjective): Because my knee kicks out so violently (hyperreflexia) and my foot is now dropping and tripping me, does this mean my spinal cord is being compressed again? Do I need an urgent new MRI of my neck or lower back?

Clinical Correlation & Pathophysiology: Clinical review and decision regarding repeat neuroimaging: Does the clinical coexistence of brisk patellar hyperreflexia (UMN) and acute foot drop (LMN) warrant repeat cervical MRI (to evaluate cord myelomalacia progression vs adjacent segment disease C4-C5 / C7-T1) and repeat lumbar MRI (to evaluate high-grade root compression / cauda equina risk)? Differential: Is the hyperreflexia residual myelomalacia amplified by lumbar cross-sensitization, or active cord re-compression?

Required Consult Action / Objective: Clinical evaluation of cord compression signs (Hoffmann's, clonus, Babinski) and decision on issuing urgent repeat MRI referrals.

CLINICIAN CONSULTATION DISPOSITION & SIGN-OFF

Prescriptions / Referrals Issued:
[ ] AFO Splint Orthotist Referral
[ ] Repeat Cervical / Lumbar MRI Referral
[ ] Pain Management / SCS Review

Administrative Documentation:
[ ] Medical Certificate (Police / Bail Compliance)
[ ] NDIS Access Request Form Part B Signed
[ ] Clinical Summary Letter on Clinic Letterhead

Clinician Signature & Date:

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Dr. Nathan Anderson | 04/11/2026